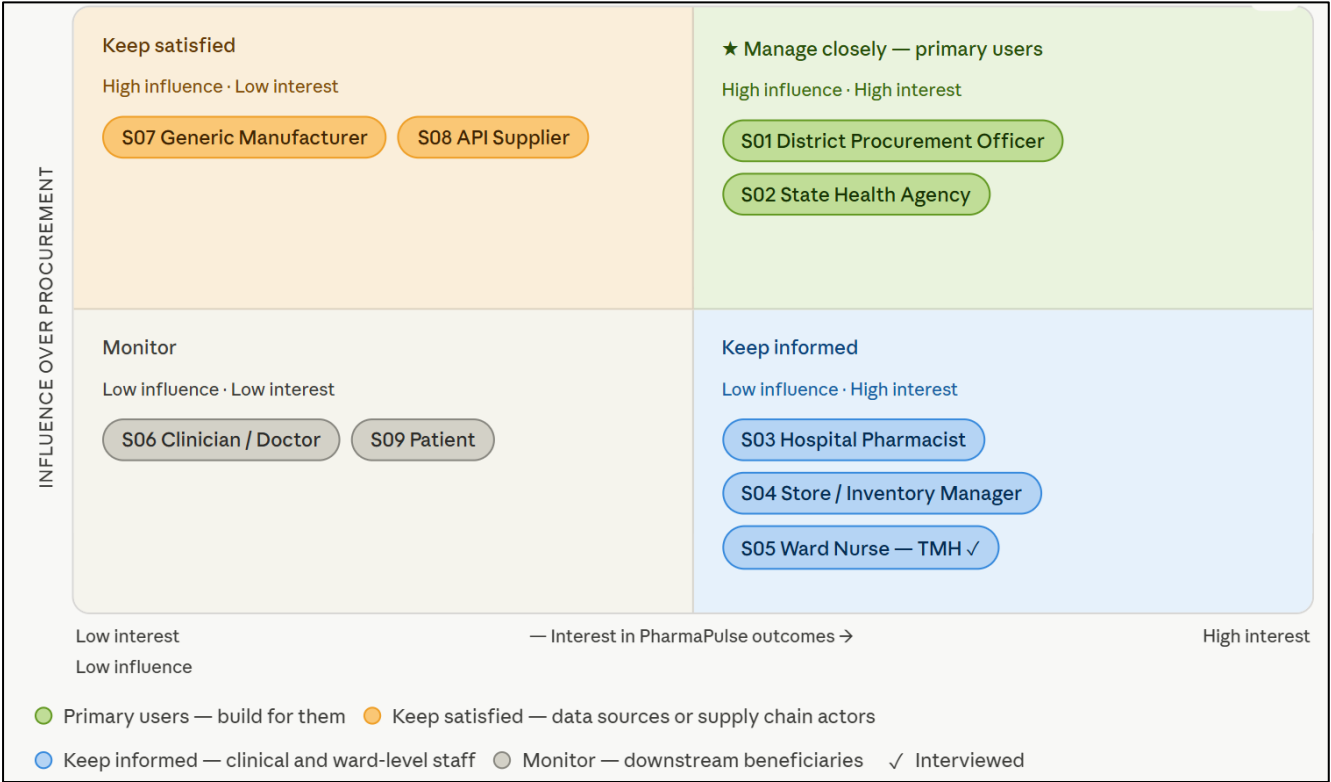


Phase 3: Process Mapping

Stakeholder	Role	Organisation Type	Interest in PharmaPulse	Influence on Procurement	Pain Point Identified	Interview Status
District Procurement Officer	Primary buyer of essential medicines	District hospital / government health dept	High — direct user of system output	High	No upstream signals; reactive decisions only	Target — Phase 2
State Health Agency Officer	Manages bulk state tenders	TNMSC / MSMSC / state procurement body	High — manages 300+ drug portfolio	Very High	Cannot pre-empt tender gaps; no risk visibility	Target — Phase 2
Hospital Pharmacist	First internal responder to shortage	Large hospital pharmacy dept	Medium — receives alerts, manages substitution	Medium	Alerts trigger too late; upstream blind spot confirmed	Indirect - via S05
Store / Inventory Manager	Manages physical and system stock	Hospital stores dept	Medium — manages ERP stock data	Low-Medium	System stock vs physical stock mismatch	Indirect - via S05

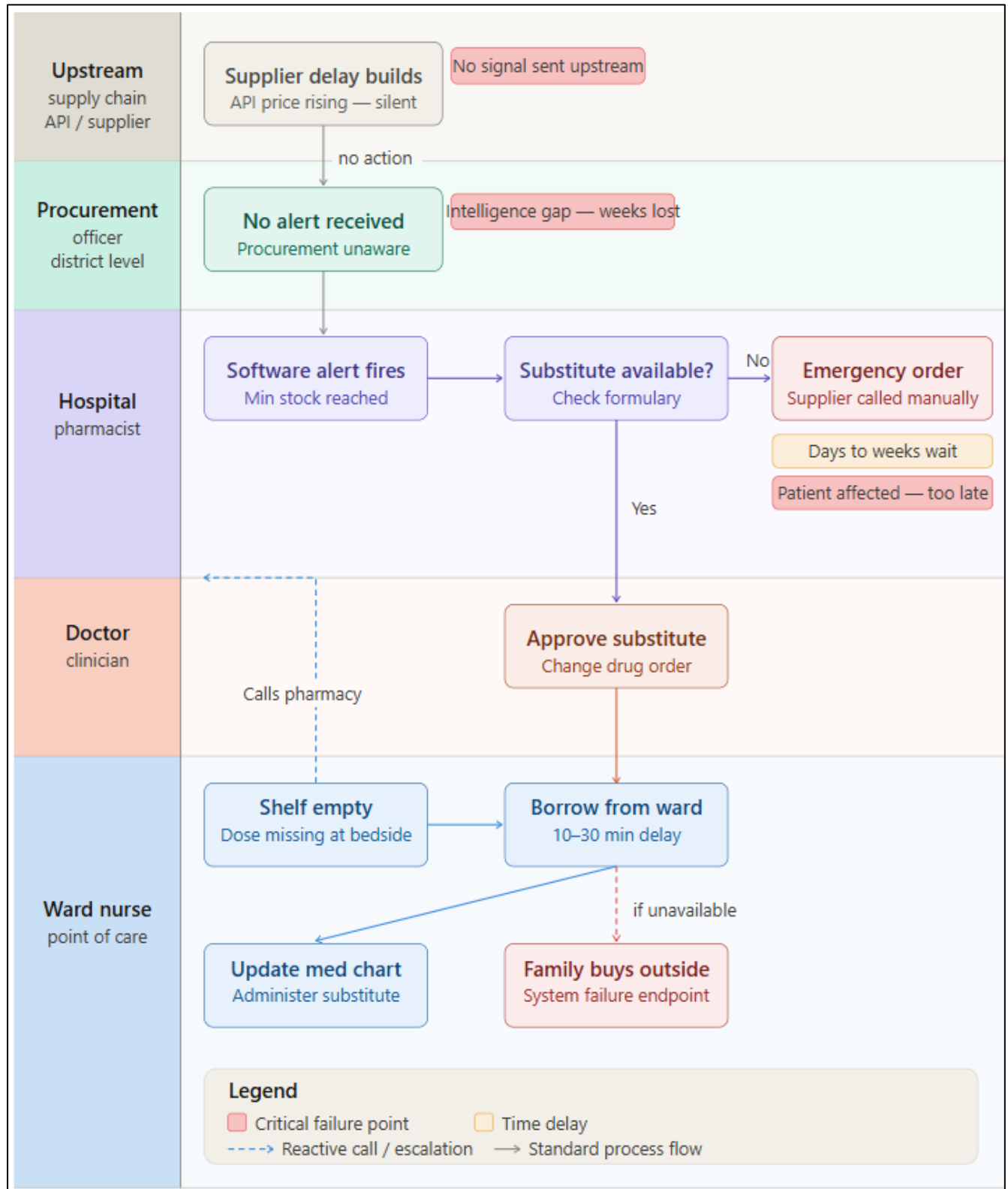
Treating Clinician / Doctor	Prescribes and approves substitutes	Hospital clinical dept	Low — approves workaround, not decision-maker	Low	Treatment protocol forced to change reactively	Target — optional
Generic Drug Manufacturer	Supplies formulations to procurement	Mid-tier pharma company	Medium — benefits from demand predictability	Medium	No visibility into downstream demand signals	Seconda
API Supplier / Manufacturer	Supplies raw material to formulators	Chemical / API manufacturing	Low direct interest	Low	Not a system user — upstream signal source only	Data sou only
Patient / Beneficiary	End recipient of medicine	Public health system	High human interest, zero system interaction	None	Medication gaps, treatment delays, out-of-pocket purchase	Represent via S05 interview

INFLUENCE-INTEREST MATRIX



AS-IS Flow:

The following swim-lane process map documents the current drug shortage response workflow across five operational actors at the ward and hospital level. The map was constructed from primary research, and identifies three critical failure points in the process.



Gap	Current State	Required State	PharmaPulse Intervention
Upstream signal visibility	Zero — no one monitors API prices or supplier health	8-12 week early warning on supply disruption	Upstream data layer — API prices, China export volume, GeM tender patterns
Procurement intelligence	Reactive — acts on empty shelf	Proactive — acts on risk score	Weekly risk dashboard for procurement officers
Cross-institution awareness	Siloed — each hospital manages alone	Connected — state-level view	State-level shortage heatmap
Decision support	Manual substitution under pressure	Pre-planned buffer stock and alternate suppliers	Recommendation engine with supplier reliability scores
Patient protection	Family sometimes buys outside	Drug always available	Shortage prevented before it reaches the ward